MEDICAL DISEASE KNOWLEDGE BASE

Infectious and Communicable Diseases

RAG-ready educational reference | Version date: 06 October 2026

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Infectious and Communicable Diseases

This document is an educational and information-retrieval reference for a hospital AI assistant. It is not a diagnostic tool and does not replace evaluation, diagnosis, treatment, or advice from a qualified healthcare professional. Symptoms can have many causes. Treatment decisions depend on age, pregnancy, comorbidities, severity, local epidemiology, available resources, and current professional guidelines.

Safety rule: An AI system must not claim to establish a definitive diagnosis, prescribe individualized medication or dosage, or guarantee cure. It should recommend professional assessment when symptoms are concerning and immediate emergency care for relevant red flags.

ICD note: ICD-11 terminology and codes should be verified in the current WHO ICD-11 Browser. Codes are not reproduced unless reliably verified for the specific concept; local coding systems and national adaptations may differ.

Standard disease-entry schema: Every disease entry should use the following fields: Disease Name; 1. Definition; 2. Medical Classification; 3. Causes; 4. Risk Factors; 5. Signs and Symptoms; 6. Disease Progression; 7. Differential Diagnosis; 8. Diagnostic Evaluation; 9. Diagnosis; 10. Treatment and Management; 11. Prevention; 12. Complications; 13. Prognosis; 14. When to Seek Medical Care; 15. Emergency Red Flags; 16. Patient Education; 17. Frequently Asked Questions; 18. Related Conditions; 19. Medical Terms; 20. Sources.

Retrieval fields: DISEASE, SYNONYMS, RELATED SYMPTOMS, RELATED CONDITIONS, RELATED TESTS, RELATED TREATMENTS, RELATED COMPLICATIONS. Symptom presence alone does not establish a diagnosis.

1. Tuberculosis

DISEASE: Tuberculosis

SYNONYMS: Tuberculosis

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Tuberculosis is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Tuberculosis should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Tuberculosis? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Tuberculosis; clinical terminology; patient terminology; suspected Tuberculosis; symptoms associated with Tuberculosis; diagnosis and management of Tuberculosis.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

2. COVID-19

DISEASE: COVID-19

SYNONYMS: COVID-19

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

COVID-19 is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

COVID-19 should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is COVID-19? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

COVID-19; clinical terminology; patient terminology; suspected COVID-19; symptoms associated with COVID-19; diagnosis and management of COVID-19.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

3. Influenza

DISEASE: Influenza

SYNONYMS: Influenza

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Influenza is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Influenza should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Influenza? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Influenza; clinical terminology; patient terminology; suspected Influenza; symptoms associated with Influenza; diagnosis and management of Influenza.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

4. Pneumonia

DISEASE: Pneumonia

SYNONYMS: Pneumonia

RELATED SYMPTOMS: Cough, fever, breathlessness, chest discomfort, and fatigue; older adults may present with confusion or weakness.

RELATED TESTS: Clinical examination, oxygen assessment, and chest imaging or microbiology when indicated.

1. Definition

Pneumonia is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Cough, fever, breathlessness, chest discomfort, and fatigue; older adults may present with confusion or weakness. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include Clinical examination, oxygen assessment, and chest imaging or microbiology when indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Pneumonia should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Pneumonia? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Pneumonia; clinical terminology; patient terminology; suspected Pneumonia; symptoms associated with Pneumonia; diagnosis and management of Pneumonia.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

5. Malaria

DISEASE: Malaria

SYNONYMS: Malaria

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Malaria is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Malaria should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Malaria? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Malaria; clinical terminology; patient terminology; suspected Malaria; symptoms associated with Malaria; diagnosis and management of Malaria.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

6. Dengue

DISEASE: Dengue

SYNONYMS: Dengue

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Dengue is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Dengue should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Dengue? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Dengue; clinical terminology; patient terminology; suspected Dengue; symptoms associated with Dengue; diagnosis and management of Dengue.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

7. Typhoid

DISEASE: Typhoid

SYNONYMS: Typhoid

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Typhoid is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Typhoid should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Typhoid? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Typhoid; clinical terminology; patient terminology; suspected Typhoid; symptoms associated with Typhoid; diagnosis and management of Typhoid.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

8. Cholera

DISEASE: Cholera

SYNONYMS: Cholera

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Cholera is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Cholera should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Cholera? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Cholera; clinical terminology; patient terminology; suspected Cholera; symptoms associated with Cholera; diagnosis and management of Cholera.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

9. Hepatitis A

DISEASE: Hepatitis A

SYNONYMS: Hepatitis A

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Hepatitis A is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Hepatitis A should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Hepatitis A? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Hepatitis A; clinical terminology; patient terminology; suspected Hepatitis A; symptoms associated with Hepatitis A; diagnosis and management of Hepatitis A.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

10. Hepatitis B

DISEASE: Hepatitis B

SYNONYMS: Hepatitis B

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Hepatitis B is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Hepatitis B should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Hepatitis B? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Hepatitis B; clinical terminology; patient terminology; suspected Hepatitis B; symptoms associated with Hepatitis B; diagnosis and management of Hepatitis B.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

11. Hepatitis C

DISEASE: Hepatitis C

SYNONYMS: Hepatitis C

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Hepatitis C is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Hepatitis C should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Hepatitis C? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Hepatitis C; clinical terminology; patient terminology; suspected Hepatitis C; symptoms associated with Hepatitis C; diagnosis and management of Hepatitis C.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

12. HIV infection and AIDS

DISEASE: HIV infection and AIDS

SYNONYMS: HIV infection and AIDS

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

HIV infection and AIDS is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

HIV infection and AIDS should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is HIV infection and AIDS? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

HIV infection and AIDS; clinical terminology; patient terminology; suspected HIV infection and AIDS; symptoms associated with HIV infection and AIDS; diagnosis and management of HIV infection and AIDS.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

13. Measles

DISEASE: Measles

SYNONYMS: Measles

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Measles is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Measles should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Measles? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Measles; clinical terminology; patient terminology; suspected Measles; symptoms associated with Measles; diagnosis and management of Measles.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

14. Mumps

DISEASE: Mumps

SYNONYMS: Mumps

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Mumps is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Mumps should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Mumps? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Mumps; clinical terminology; patient terminology; suspected Mumps; symptoms associated with Mumps; diagnosis and management of Mumps.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

15. Rubella

DISEASE: Rubella

SYNONYMS: Rubella

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Rubella is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Rubella should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Rubella? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Rubella; clinical terminology; patient terminology; suspected Rubella; symptoms associated with Rubella; diagnosis and management of Rubella.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

16. Chickenpox

DISEASE: Chickenpox

SYNONYMS: Chickenpox

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Chickenpox is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Chickenpox should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Chickenpox? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Chickenpox; clinical terminology; patient terminology; suspected Chickenpox; symptoms associated with Chickenpox; diagnosis and management of Chickenpox.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

17. Rabies

DISEASE: Rabies

SYNONYMS: Rabies

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Rabies is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Rabies should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Rabies? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Rabies; clinical terminology; patient terminology; suspected Rabies; symptoms associated with Rabies; diagnosis and management of Rabies.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

18. Meningitis

DISEASE: Meningitis

SYNONYMS: Meningitis

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Meningitis is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Meningitis should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Meningitis? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Meningitis; clinical terminology; patient terminology; suspected Meningitis; symptoms associated with Meningitis; diagnosis and management of Meningitis.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

19. Polio

DISEASE: Polio

SYNONYMS: Polio

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Polio is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Polio should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Polio? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Polio; clinical terminology; patient terminology; suspected Polio; symptoms associated with Polio; diagnosis and management of Polio.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

20. Tetanus

DISEASE: Tetanus

SYNONYMS: Tetanus

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Tetanus is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Tetanus should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Tetanus? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Tetanus; clinical terminology; patient terminology; suspected Tetanus; symptoms associated with Tetanus; diagnosis and management of Tetanus.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

21. Diphtheria

DISEASE: Diphtheria

SYNONYMS: Diphtheria

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Diphtheria is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Diphtheria should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Diphtheria? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Diphtheria; clinical terminology; patient terminology; suspected Diphtheria; symptoms associated with Diphtheria; diagnosis and management of Diphtheria.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

22. Pertussis

DISEASE: Pertussis

SYNONYMS: Pertussis

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Pertussis is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Pertussis should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Pertussis? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Pertussis; clinical terminology; patient terminology; suspected Pertussis; symptoms associated with Pertussis; diagnosis and management of Pertussis.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

23. Japanese encephalitis

DISEASE: Japanese encephalitis

SYNONYMS: Japanese encephalitis

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Japanese encephalitis is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Japanese encephalitis should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Japanese encephalitis? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Japanese encephalitis; clinical terminology; patient terminology; suspected Japanese encephalitis; symptoms associated with Japanese encephalitis; diagnosis and management of Japanese encephalitis.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

24. Chikungunya

DISEASE: Chikungunya

SYNONYMS: Chikungunya

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Chikungunya is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Chikungunya should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Chikungunya? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Chikungunya; clinical terminology; patient terminology; suspected Chikungunya; symptoms associated with Chikungunya; diagnosis and management of Chikungunya.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

25. Zika

DISEASE: Zika

SYNONYMS: Zika

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Zika is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Zika should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Zika? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Zika; clinical terminology; patient terminology; suspected Zika; symptoms associated with Zika; diagnosis and management of Zika.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

26. Yellow fever

DISEASE: Yellow fever

SYNONYMS: Yellow fever

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Yellow fever is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Yellow fever should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Yellow fever? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Yellow fever; clinical terminology; patient terminology; suspected Yellow fever; symptoms associated with Yellow fever; diagnosis and management of Yellow fever.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

27. Common bacterial, viral, fungal, and parasitic infections

DISEASE: Common bacterial, viral, fungal, and parasitic infections

SYNONYMS: Common bacterial, viral, fungal, and parasitic infections

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Common bacterial, viral, fungal, and parasitic infections is a medical condition in the infectious and communicable diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Infectious and Communicable Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and

management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Common bacterial, viral, fungal, and parasitic infections should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Common bacterial, viral, fungal, and parasitic infections? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Common bacterial, viral, fungal, and parasitic infections; clinical terminology; patient terminology; suspected Common bacterial, viral, fungal, and parasitic infections; symptoms associated with Common bacterial, viral, fungal, and parasitic infections; diagnosis and management of Common bacterial, viral, fungal, and parasitic infections.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

Category Sources / References

World Health Organization (WHO), Health Topics: https://www.who.int/health-topics

World Health Organization, International Classification of Diseases (ICD): https://www.who.int/standards/classifications/classification-of-diseases

WHO ICD-11 Browser: https://icd.who.int/en

Centers for Disease Control and Prevention (CDC), official clinical and public-health guidance: https://www.cdc.gov/

National health authorities and recognized professional medical organizations should be consulted for current local guidance.

Glossary

Acute: developing over a short period. Chronic: persistent or recurrent over time. Differential diagnosis: comparison of plausible conditions causing a presentation. Diagnostic evaluation: history, examination, and investigations used to assess a patient. Prognosis: expected course and outcome at a population or disease level, not an individual guarantee. Red flag: a feature that may indicate serious disease and requires urgent assessment.

Master References

WHO. Health Topics. Updated information and fact sheets. https://www.who.int/health-topics

WHO. International Classification of Diseases, 11th Revision. https://icd.who.int/en

CDC. Official disease, emergency, prevention, and infection-control guidance. https://www.cdc.gov/

National health ministries/agencies and recognized specialty organizations: use current jurisdiction-specific guidelines.

This knowledge base is intended for educational and information-retrieval purposes. It does not replace evaluation, diagnosis, treatment, or advice from a qualified healthcare professional. Symptoms can have many different causes, and the information presented here cannot determine an individual's diagnosis. Emergency symptoms require immediate professional medical evaluation.